



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه بهاء عبدالغفار عبدالهادي

المحافظة : القاهرة الحي / المدينة : بولاق ابو العلا

رقم البطاقة : 27811160103469 رقم التليفون : 01155328437

العمر (تاريخ الميلاد) : 16-11-1978 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____

المقر بما فيه : _____

التوقيع : (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض

بالصبغة [11127]

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة : _____

الوظيفة : _____

الوظيفة : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي : _____

خطة العلاج : _____

المستشفى : _____

جهة العلاج : _____

ملاحظات : _____

قسم : _____

نفقات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: فاطمة بهاء عبدالغفار عبدالهادي
National ID: 27811160103469



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

مستشفى
روسيدي
مركز التخصصات

9-12



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للتنظيم
الطبي

09/13/2026

MDT FORM

Name:	نظف بهاء عبدالعظيم	MRN:	13059
Age:	48 Y	ID:	27811160103469
Menopausal Sta	PRE	Phone:	01155328437
NRS SCORE:		Marital Status:	M
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	NEW CASE

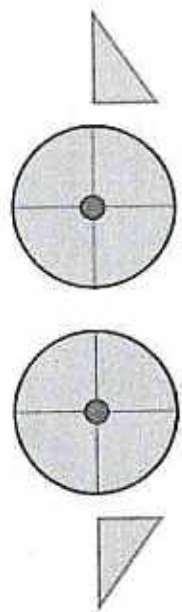
Clinical Data:

BX:	IDC G3							
ER:	8/8	PR:	0/8	HER2:		SISH NO1	KT67:	

R

L

Sonomammography:
CESM: LEFT BREAST UOQ SPICULATED LESION WITH NON MASS COLLECTIVELY MEASURING 11*5 CM
NORMAL SKIN THICKNESS
NORMAL NAC
NO PATHOLOGICAL AXILLARY LNS
BX: IDC G3 ER 8/8 PR 0/8 HER2 +2 SISH: NOT AMPLIFIED
CLINICALLY : LEFT BREAST UOQ LARGE MASS WITH SKIN ATTACHMENT



Mets Workup:
PET CT : NO OTHER FDG ACTIVE LESIONS

T N M

History:

Recommendations:

MDT Outcome & Plan
FOR CT CAP IN GRI
PORT-CTH
ECHO
CLIPPING + BRACKTING
NA CTH FOR DOWNSIZING AC X4 TTX 4 US EVERY 2 CYCES
THEN WLE+LN ACCORDING

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د محمد كمال	د محمد توفيق				

Signature

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمة
بهاء عبدالغفار عبدالهادي
٣٤ شارع العتلات
بولاق - القاهرة



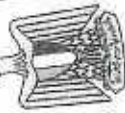
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٢٧٨١١١٦٠١٠٣٤٦٩

IL5802552



٢٠٢٠/٠٧



٢٧٨١١١٦٠١٠٣٤٦٩
محمية حرة

أنثى

مسلمة

متزوجة
محمي الدين شعبان احمد عبدالهادي اللجار

البطاقة سارية حتى ٢٠٢٧/٠٧/٠٧



Patient Name	فاطمة بهاء عبدالغفار	Case No.	357695
Sex/Age	Female 47 Y	Patient No.	25811
Registered Date	12/8/2026 01:18:16 PM	Referred By Dr.	Out Patient
Validation Date	06/9/2026 12:43:08 PM	Branch	Gustave Roussy
Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

Breast Slide Revision

Clinical Data: Left breast 2 O'clock lesion (BIRADs 4c).

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (2339126). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2& SISH).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Tumor infiltrating lymphocytes (TILs): Moderate.

IPT:

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Positive (8/8).
- PR: Negative (0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group II.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).



غرفة رقم (4) - الدور الأول - مبنى رشيد الشاذلي - معمل الباثولوجيا - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadoma - Cairo, Egypt

Patient Name	فاطمة بهاء عبدالغفار	Case No.	357695
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Validation Date	06/9/2026 12:43:08 PM	Branch	Gustave Roussy
Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

****Scoring system: "ER, PR"**
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**
Reference: ASCO-CAP Her2 Test Guideline Recommendations.IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the

the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells. invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4. (Recounting give the same result).
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.
- 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



خريطة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمه
بهام عبدالغفار عبدالهادي
٣٤ شارع العائلات
بولاق - القاهرة

١٩٧٨/١



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مهامية خرد



أشئ مسلمة متروجة
محمي الدين شعبان احمد عبدالهادي النجار
البطاقة سارية حتى ٢٠٢٧/٠٧/٠٧



BQHEYA

Radiology Department

قسم الأشعة الإشعاعية

PATIENT NAME: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY

PATIENT ID: 10126282501

GENDER: Female

AGE: 47 y

DATE : 04-Jul-2026 01:25 PM

ORDER: 3939421,3939422

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Heterogeneously dense breasts (ACR c).

Left breast:

- Left UOQ posterior third dense spiculated mass lesion seen measuring 5.2x4.7cm, 10 cm distance from nipple at CC view.
- Complementary US shows 2:00 zone c hypoechoic vertically oriented spiculated mass lesion seen about 2mm deep to overlying skin.
- No pathologically enlarged axillary level I, II or III lymph nodes.
- Clear supraclavicular regions bilaterally.

Right breast:

- No spiculated mass lesions or grouped microcalcifications.
- UOQ two nearby parallel hypoechoic circumscribed lesions at 10:00 measuring 1.6x0.4 and 1x0.6cm.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Left breast UOQ malignant looking lesion (BIRADS 4); for histopathological correlation.
- Right breast UOQ benign looking nodules (BIRADS 3).

PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y

Report version 1, 04-Jul-2026 02:52 PM



RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Ghada Ali, MSc

04-Jul-2026 02:52 PM

PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y
Report version 1, 04-Jul-2026 02:52 PM



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MRN: 10126282501
Patient Name: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
IC No.: 27811160103469
Location: External Clinic

D.O.B: 16/11/1978
Sex: Female

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id 2607060199
Accession No SUR-26-2339
Sample Type Tissue

Collection Date 06/07/2026 01:22 PM
Received Date 06/07/2026 01:22 PM
Doctor Zeinab Amr Mostafa Kamal

Clinical Information

Left breast lesion at 2 O'clock (BIRADS 4c).

Gross Picture

Multiple grayish tissue cores 0.8-1.3 cm long, totally submitted.

Microscopic Picture

A case of invasive duct carcinoma, grade 3. No DCIS component. scattered lymphovascular emboli are detected . Stroma is desmoplastic with mild inflammatory cell reaction.

Frozen Diagnosis

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, NST, grade 3.

Laterality

Left

Specimen type:

U/S guided core biopsy.

Location

2 O'clock

Lesion Size

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):

3

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

Histological grade (1-3):

3

Peritumoural lymphovascular invasion

Present

Involvement of muscle

DCIS in specimen

Absent

Diagnosis

Left breast 2 O'clock lesion, U/S guided core biopsy:

Invasive duct carcinoma, grade 3.

Comments

ER, PR & HER-2 are available (upon request).

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Ahmed Ali. MD.

Co-signed by:

Dr. Marwa Mahmoud Abd Elkader, FEBP, IFCAP.

Reported and electronically endorsed by

Marwa Mahmoud Abdelkader Nada

**Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP**

Validated on 11/07/2026 09:47 PM

Print Date: 23/07/2026 11:51 AM

Page Number: 2 of 2

MRN: 10126282501
Patient Name: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
IC No.: 27811160103469
Location: Surgical Oncology

D.O.B: 16/11/1978
Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2607130195
Accession No IMM-26-2798
Sample Type Paraffin Block

Collection Date 13/07/2026 12:12 PM
Received Date 13/07/2026 12:12 PM
Doctor Maha Mohamed Wagdy Omar

Immunohistochemical Report

pathology number
2339/26

Clinical Data
Left breast cancer

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong).
Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:
left

Diagnosis :

Left breast cancer:

ER : positive, strong nuclear reaction in 100% of tumor cells (8/8).

PR : Negative (0/8)

Signature

Esraa Esmat, MD

Reported and electronically endorsed by

Esraa esmat mohamed abdelmonem mohamed

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

Validated on 18/07/2026 07:19 PM

Print Date: 23/07/2026 11:51 AM

Page Number: 2 of 2

MRN: 10126282501
Patient Name: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
IC No.: 27811160103469
Location: Surgical Oncology

D.O.B: 16/11/1978
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2607130194
Accession No IMM-26-2797
Sample Type Paraffin Block

Collection Date 13/07/2026 12:12 PM
Received Date 13/07/2026 12:12 PM
Doctor Maha Mohamed Wagdy Omar

Immunohistochemical Report

pathology number
2339/26

Clinical Data

Left breast cancer

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:

left

Laterality :

Left

Her2 Result

Equivocal [score 2+]. Recommended For SISH

ALHADAY.

Comment :

STSH is available upon request

Signature

Esraa Esmat, MD

Reported and electronically endorsed by

Esraa esmat mohamed abdelmonem mohamed

**Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP**

Validated on 18/07/2026 07:20 PM

Print Date:

23/07/2026 11:51 AM

Page Number: 2 of 2

PATIENT NAME: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
GENDER: Female
DATE : 21-Jul-2026 10:14 AM

PATIENT ID: 10126282501
AGE: 47 y
ORDER: 3951711

WHOLE BODY FDG PET-CT EXAMINATION

CLINICAL BACKGROUND:

- 47 year-old female patient
- Presented with left breast cancer
- Weight = 83 Kg
- BL glucose level = 113 mg/dl

REASON OF STUDY:

- Initial staging study

TECHNIQUE:

- Approximately one hour after IV injection of the F-18-FDG 5.8 mCi, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- No IV contrast was given.
- Data was processed and displayed according to department protocol.
- Calculation of SUVmax was done at region of interest.
- Reference hepatic activity (SUVmax)~ 3.52

FINDINGS:

Reviewing PET, CT and fused PET-CT images:-

I- Breasts and axillary regions:

- Left breast metabolically active (FDG avid) UOQ irregular soft tissue mass is seen measuring about 4.1 x 3.9 cm, SUVmax~ 17.02. It is seen abutting the skin with minimal overlying skin thickening.
- Few small left axillary metabolically inactive lymph nodes the largest measuring 1.6 x 0.6 cm.
- Right breast no FDG avid focal sizable nodules/masses.

PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y

Report version 3, 22-Jul-2026 11:02 AM

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مركز ب.هية
4 شارع عدوية أول شارع الهدم جوار دارى, الحرة
مستشفى ب.هية الشيخ زايد
ع: الحرة الأولى - الشيخ زايد رقم 4000



- No significant FDG uptake in right axillary lymph nodes.

II. Head and neck:

- Diffuse symmetrical tracer uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- No metabolically active (FDG avid) cervical lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Normal appearance of the aerodigestive air column with no parapharyngeal masses.
- Normal appearance of the vocal cords, both pyriform sinuses and ary-epiglottic folds. No glottic, supra- or infraglottic masses.
- Intact laryngeal skeleton.
- Heterogenous thyroid gland showing multiple small bilobar low grade metabolically active hypodense nodules largest at left measuring 1.2 x 0.8 cm, SUVmax~ 3.58.
- Right maxillary and sphenoid sinusitis.

III. Chest:

- No metabolically active (FDG avid) pulmonary nodules.
- No metabolically active mediastinal or hilar lymph nodes.
- No metabolically active pleural effusion or masses.
- Average cardiac size. No pericardial effusion detected.

IV. Abdomen & pelvis:

- The liver is average in size with homogenous CT texture. No definite metabolically active (FDG avid) focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of the pancreas, both kidneys and both suprarenal (on non contrast CT basis) with no FDG avid focal lesions.
- No metabolically active abdominal or pelvic lymphadenopathy.
- Under filling of the urinary bladder.
- Right adnexal low grade metabolically active simple cyst measuring 2.3 x 1.6 cm, SUVmax~ 4.19 likely functional.
- IUCD is seen.
- No ascites.

V. Scanned axial and proximal appendicular skeleton:

- No metabolically active (FDG avid) osseous lesions.
- Left sacral alium and left femoral head small metabolically inactive sclerotic lesions likely bone islands.
- Spondylodegenerative changes with sclerosis of the VEPS likely

ATTENT: FATIMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y
port version 3, 22-Jul-2026 11:02 AM

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16602

مركز فهدية البهية
4 شارع سعودية بول شارع الوحدة بجوار نادي الجزيرة
مستشفى فهدية البهية المشيخ وكه
4- الثاني الأول - الشيخ زايد عدم كلية الهندسة



degenerative.

OPINION:

FDG-PET/CT study of pathologically proven left breast cancer at initial staging showing:

- Left breast metabolically active neoplastic lesion.
- No other metabolically active (FDG avid) lesions in the rest of the surveyed body.

Reported By,
Dr. Ali Saber, MSc
Dr. Salwa Yahya, MD
Prof. Mai Amr, MD

22-Jul-2026 11:02 AM

PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y
version 3, 22-Jul-2026 11:02 AM

Medical Department
Dr. Ali Saber
22-Jul-2026 11:02 AM



GENDER: Female

PATIENT ID: 10126282501

AGE: 47 y

ORDER: 3951690

INDICATION:

- Left carcinoma

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- IV low osmolar non-ionic contrast medium infusion 100 ml

REPORT:

- Contrast Enhanced Spectral Mammography showed mild symmetrical back ground parenchymal enhancement.
- Left UOQ spiculated mass is seen showing marked heterogeneous mass enhancement measured 5.8x3.5 cm, anterior and inferior enhancing nodules and mass enhancement are seen. The mass & non mass are distributed over an area measuring 11x5 cm, the process being 4 cm from the nipple
- No right breast mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal nipple areola complexes.

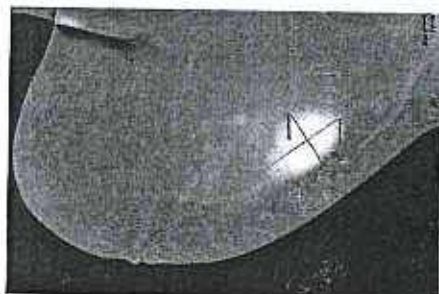
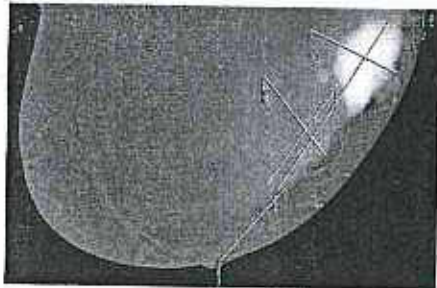
OPINION:

- Left breast UOQ multifocal carcinoma (BIRADS 6).
- No right breast pathological mass or non mass enhancement (BIRADS 1)

Reported By,

Dr. Rasha Wessam, MD 13-Jul-2026 02:33 PM

PATIENT: FATMA BAHAA ABD EL SHAFFAR ABD ALHADAY, 10126282501, Female, 47 y
Report version 1, 13-Jul-2026 02:33 PM



PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y
Report version 1, 13-Jul-2026 02:33 PM

Page 2 of 2



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16502

مركز جودة الحياة
مشاريع تطوير البنية التحتية
مشاريع تطوير البنية التحتية
مشاريع تطوير البنية التحتية
مشاريع تطوير البنية التحتية





جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة

خطاب إدارى

الرقم القومى	٢٧٨١١٦٠١٠٣٤٦٩
إسم المواطن	فاطمه بهاء عبدالغفار عبدالهادي

رقم الطلب	٢٧٩٥٩١٣١	تاريخ اللجنة ٢٠٢٦-٠٩-٢٠
الحالة	خطاب إدارى	
نص الخطاب	نوافي بالموقف من التأمين الصحي محتوم بختم التمسر (وليس التأمين الاجتماعى) ويرفق مع بطاقة الرقم القومى واضحه وساريه	
جهة الإرسال	٧٠٧٤ / ٧٠٧٤ / ٢٨٦٦٤	جوسناف روسي، انتر ناشيونال - مصر (BPE) *

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تعبير أفي ٢٠-٠٩-٢٠٢٦
٧٠٧٤ / ٧٠٧٤ / ٢٨٦٦٤



رسوم الاستعلام 20 جنية

تاريخ الاصدار : 2026-9-24

جهة الاصدار : فرع القاهرة

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للائقاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "للقوى العاملة والمعاشات والارامل " و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسى " وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسله من التأمينات الاجتماعيه
وطبقا لقاعدة البيانات المرسله من الضمان الاجتماعي
والبحت تبين ان

السيد (ة) فاطمه بهاء عبدالغفل بطاقة رقم 27811160103469
تليفون رقم: 01155328437

وطبقا الى فصل او استقاله او انتهاء مدة خدمة
يعتبر من الفئات غير المنتفعه بالتأمين الصحي صدر هذا الخطاب دون الدس مضمون له على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعه : 2026-9-24

الموظف المختص
سماح محمود



مسلسل 2088561



الخطيب ساري لمدة ثلاثة أشهر من تاريخ الاصدار إلا في حالة تغيير حله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التضامن الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية للنظم المعلومات والتحول الرقمي

مستشفى
روسيدي
مصر الجديدة



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للتنظيم
الطبي

09/13/2026

MDT FORM

Name:	ناتق بيه جابر جليل	MRN:	13059
Age:	48 Y	ID:	27811160103469
Menopausal Sta	PRE	Phone:	01155328437
NRS SCORE:		Marital Status:	M
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	NEW CASE

Clinical Data:

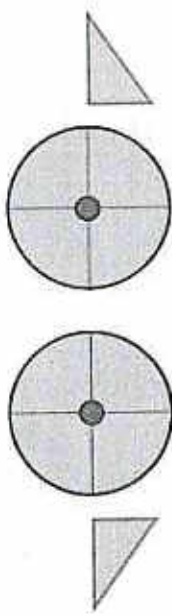
BX:	IDC G3
ER:	PR: 0/8
	HER2: SISH NO1
	KT67:

R

L

Sonomammography:

CEM: LEFT BREAST UOQ SPICULATED LESION WITH NON MASS
COLLECTIVELY MEASURING 11*5 CM
NORMAL SKIN THICKNESS
NORMAL NAC
NO PATHOLOGICAL AXILLARY LNS
BX: IDC G3 ER 8/8 PR 0/8 HER2 +2 SISH: NOT AMPLIFIED
CLINICALLY : LEFT BREAST UOQ LARGE MASS WITH SKIN ATTACHMENT



T N M

Mets Workup:

PET CT : NO OTHER FDG ACTIVE LESIONS

History:

Recommendations:

MDT Outcome & Plan

FOR CT CAP IN GRI
PORT-CTH
ECHO
CLIPPING + BRACKTING
NA CTH FOR DOWNSIZING AC X4 TTX 4 US EVERY 2 CYCES
THEN WLE+LN ACCORDING

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد كمال	د. محمد توفيق				

Signature

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمة
بهاء عبدالغفار عبدالهادي
٣٤ شارع العتلات
بولاق - القاهرة

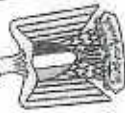
١٩٧٨/١١/١٦

٢٧٨١١١٦٠١٠٣٤٦٩

IL5802552



٢٠٢٠/٠٧



٢٧٨١١١٦٠١٠٣٤٦٩
محمية حرة

أنقى

مصلحة

متروجة
محمي الدين شعبان احمد عبدالهادي اللجار
البطاقة سارية حتى ٢٠٢٧/٠٧/٠٧



Patient Name	فاطمة بهاء عبدالغفار	Case No.	357695
Sex/Age	Female 47 Y	Patient No.	25811
Registered Date	12/8/2026 01:18:16 PM	Referred By Dr.	Out Patient
Validation Date	06/9/2026 12:43:08 PM	Branch	Gustave Roussy
Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

Breast Slide Revision

Clinical Data: Left breast 2 O'clock lesion (BIRADs 4c).

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (2339126). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2& SISH).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Tumor infiltrating lymphocytes (TILs): Moderate.

IPT:

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Positive (8/8).
- PR: Negative (0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group II.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).



Patient Name	فاطمة بهاء عبدالغفار	Case No.	357695
Sex/Age	Female 47 Y	Patient No.	25811
Registered Date	12/8/2026 01:18:16 PM	Referred By Dr.	Out Patient
Validation Date	06/9/2026 12:43:08 PM	Branch	Gustave Roussy
Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations.IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the

the invasive tumor cells or complete and circumferential membrane staining that is complete, intense > 10% of tumor cells. invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4. (Recounting
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.
- 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



شقة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



جمهورية مصر العربية
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المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة

خطاب إدارى

الرقم القومى	٢٧٨١١٦٠١٠٣٤٦٩
إسم المواطن	فاطمه بهاء عبدالغفار عبدالهادي

رقم الطلب	٢٧٩٥٩١٣١	تاريخ اللجنة ٢٠٢٦-٠٩-٢٠
الحالة	خطاب إداري	
نص الخطاب	نوافي بالموقف من التأمين الصحي محتوم بختم التمسر (وليس التأمين الاجتماعي) ويرفق مع بطاقة الرقم القومي واضحه وساريه	
٧٠٧٤ / ٧٠٧٤ / ٢٨٦٦٤		جهة الإرسال جوسناف روسي انتر ناسيونال - مصدر (BPE) *

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تعبير أفي ٢٠-٠٩-٢٠٢٦
٧٠٧٤ / ٧٠٧٤ / ٢٨٦٦٤



رسوم الاستعلام 20 جنية

تاريخ الاصدار : 2026-9-24

جهة الاصدار : فرع القاهرة

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للائقاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "للقوى العاملة والمعاشات والارامل " و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسى " وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسله من التأمينات الاجتماعيه
وطبقا لقاعدة البيانات المرسله من الضمان الاجتماعي
والبحت تبين ان

السيد (ة) فاطمه بهاء عبدالغفل بطاقة رقم 27811160103469
تليفون رقم: 01155328437

وطبقا الى فصل او استقاله او انتهاء مدة خدمة
يعتبر من الفئات غير المنتفعه بالتأمين الصحي صدر هذا الخطاب دون الدس مضمون له على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعه : 2026-9-24

الموظف المختص
سماح محمود



مسلسل 2088561



الخطيب ساري لمدة ثلاثة أشهر من تاريخ الاصدار إلا في حالة تغيير حالة المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التأمين الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية للنظم المعلومات والتحول الرقمي

مستشفى
روسيدي
مصر



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للتنظيم
الطبي

09/13/2026

MDT FORM

Name:	ناتق بيه جلال جلال	MRN:	13059
Age:	48 Y	ID:	27811160103469
Menopausal Sta	PRE	Phone:	01155328437
NRS SCORE:		Marital Status:	M
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	NEW CASE

Clinical Data:

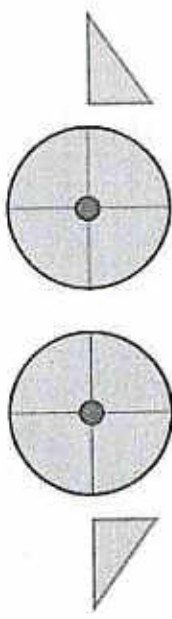
BX:	IDC G3
ER:	8/8
PR:	0/8
HER2:	
SISH NO1	
KT67:	

R

L

Sonomammography:

CEM: LEFT BREAST UOQ SPICULATED LESION WITH NON MASS
COLLECTIVELY MEASURING 11*5 CM
NORMAL SKIN THICKNESS
NORMAL NAC
NO PATHOLOGICAL AXILLARY LNS
BX: IDC G3 ER 8/8 PR 0/8 HER2 +2 SISH: NOT AMPLIFIED
CLINICALLY : LEFT BREAST UOQ LARGE MASS WITH SKIN ATTACHMENT



TNM

Mets Workup:

PET CT : NO OTHER FDG ACTIVE LESIONS

History:

Recommendations:

MDT Outcome & Plan

FOR CT CAP IN GRI
PORT-CTH
ECHO
CLIPPING + BRACKTING
NA CTH FOR DOWNSIZING AC X4 TTX 4 US EVERY 2 CYCES
THEN WLE+LN ACCORDING

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد كمال	د. محمد توفيق				

Signature

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمة
بهاء عبدالغفار عبدالهادي
٣٤ شارع العتلات
بولاق - القاهرة

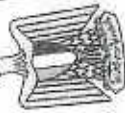
١٩٧٨/١١/١٦

٢٧٨١١١٦٠١٠٣٤٦٩

IL5802552



٢٠٢٠/١٠/٧



٢٧٨١١١٦٠١٠٣٤٦٩
محمية حرة

أنثى

مسلمة

متزوجة
محمي الدين شعبان احمد عبدالهادي اللجار

البطاقة سارية حتى ٢٠٢٧/١٠/٧



Patient Name	فاطمة بهاء عبدالغفار	Case No.	357695
Sex/Age	Female 47 Y	Patient No.	25811
Registered Date	12/8/2026 01:18:16 PM	Referred By Dr.	Out Patient
Validation Date	06/9/2026 12:43:08 PM	Branch	Gustave Roussy
Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

Breast Slide Revision

Clinical Data: Left breast 2 O'clock lesion (BIRADs 4c).

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (2339126). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2& SISH).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Present.
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- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Positive (8/8).
- PR: Negative (0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group II.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).



Patient Name	فاطمة بهاء عبدالغفار	Case No.	357695
Sex/Age	Female 47 Y	Patient No.	25811
Registered Date	12/8/2026 01:18:16 PM	Referred By Dr.	Out Patient
Validation Date	06/9/2026 12:43:08 PM	Branch	Gustave Roussy
Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

****Scoring system: "ER, PR"**
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**
Reference: ASCO-CAP Her2 Test Guideline Recommendations.IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells.

IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells. IHC groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4. (Recounting give the same result).
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.
- 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



شقة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt